CLINICAL STUDY PROTOCOL ? AMENDMENT 1
Study ID: TX-2026-01 | Phase: III | Indication: Moderate-to-Severe Chronic Inflammatory Disease
Protocol Version: 2.0 (Amendment 1) | Effective Date: June 30, 2026
Sponsor: TrialSync Clinical Therapeutics, Inc.

RATIONALE FOR PROTOCOL AMENDMENT

Following review of preliminary blinded pharmacokinetic/pharmacodynamic modeling and evolving regulatory guidance on durability of response, the primary observation window has been extended to capture long-term therapeutic remission and prevent premature truncation of treatment effect.

1. STUDY SYNOPSIS AND OBJECTIVES

1.1 Primary Objective

The primary objective of this study is to evaluate the sustained clinical efficacy of TS-101 compared with placebo in achieving durable disease remission.

1.2 Secondary Objectives

- Assess long-term safety, tolerability, and immunogenicity of TS-101.
- Measure sustained patient-reported quality of life over extended follow-up.

2. STUDY DESIGN AND POPULATION

This multicenter, randomized, double-blind Phase III investigation continues enrollment of approximately 240 adult participants across 18 clinical sites. Additional visit windows have been incorporated to accommodate extended treatment and monitoring.

3. SCHEDULE OF ASSESSMENTS AND AMENDED ENDPOINT
Participants will attend scheduled clinic visits for screening, baseline randomization, and regular monitoring visits.

section: endpoint
Primary endpoint assessment occurs at week 36.
The primary efficacy estimand is the proportion of participants achieving sustained clinical remission (defined as a reduction in composite inflammatory score >= 50% from baseline) without rescue therapy. All primary statistical inferences will be conducted on the intent-to-treat (ITT) population at this extended time point.

4. SAFETY MONITORING AND REGULATORY GOVERNANCE
Adverse events and serious adverse events will continue to be monitored through the extended safety follow-up window. The DMC oversight charter remains active throughout the expanded treatment period.

5. HISTORICAL BENCHMARK CITATION
Historical benchmark CSR-2024-09 remains cited as prior context for the early 24-week milestone, but subsequent operational and statistical plans must be aligned with the amended 36-week primary endpoint.